Clinical Practice Guideline: Management of Hypertension in Primary Care (Abridged)

Guideline ID: DEMO-HTN-2020-A **Version:** 1.0 (demonstration abridgement) **Effective Date:** March 2020 **Issuing Body:** Adapted for demonstration from the VA/DoD Clinical Practice Guideline for the Diagnosis and Management of Hypertension in the Primary Care Setting, Version 5.0 (March 2020)

1. Scope and Purpose

1.1 Clinical Condition

This guideline covers the treatment of adults with a confirmed diagnosis of hypertension in the primary care setting. It is a demonstration abridgement: it keeps a small number of the source guideline's recommendations verbatim in substance, with their original recommendation numbers, and omits the rest.

1.2 Target Population

Adults aged 30 years and older with hypertension who are managed in primary care.

1.3 Exclusions

Hypertensive emergency, secondary hypertension, pregnancy, and resistant hypertension are outside the scope of this abridgement.

2. Background

Blood pressure goals in the source guideline depend on age and the presence of type 2 diabetes. First-line drug therapy is any of three classes, and home blood pressure self-monitoring improves control only when it is paired with a co-intervention such as tailored medication titration or education.

3. Recommendations

3.1 Blood Pressure Goals (source recommendations 6–9)

For all patients, including those with type 2 diabetes, the systolic goal is below 130 mmHg (Weak for). Where a less stringent goal is chosen by clinical judgment or patient preference, the goal is at least below 150 mmHg for patients aged 60 years and over (Strong for) and at least below 140 mmHg for patients aged 60 years and over with type 2 diabetes (Strong for). For patients aged 30 years and over, the diastolic goal is below 90 mmHg (Weak for).

Decision Table 1: Blood Pressure Goal

Age 60 or over	Has Type 2 Diabetes	Systolic Goal (mmHg)	Diastolic Goal (mmHg)
No	No	< 130	< 90
No	Yes	< 130	< 90
Yes	No	< 150	< 90
Yes	Yes	< 140	< 90

Key principles:

1 A blood pressure is **above goal** when the systolic value is at or above the systolic goal OR the diastolic value is at or above the diastolic goal.

1 The rows for patients aged 60 and over state the less stringent goal; lowering further toward 130 mmHg systolic has added benefit.

3.2 Drug Therapy (source recommendations 23, 25, 26)

A thiazide-type diuretic, a calcium channel blocker, or either an ACE inhibitor or an angiotensin II receptor blocker is recommended as primary pharmacologic therapy (Strong for). For patients aged 65 years and over, a thiazide-type diuretic is suggested (Weak for). More than one of an ACE inhibitor, an angiotensin II receptor blocker, or a direct renin inhibitor must not be used together in the same patient (Strong against). Patients whose systolic or diastolic pressure is more than 20/10 mmHg above goal are unlikely to reach goal with monotherapy and should start two agents.

Decision Table 2: Therapy Action

Above Goal	Margin Above Goal More Than 20/10 mmHg	Age 65 or over	Action
No	No	Any	Continue current therapy
Yes	No	No	Start or add one first-line agent
Yes	No	Yes	Start or add a thiazide-type diuretic
Yes	Yes	Any	Start or add two first-line agents

3.3 Home Blood Pressure Self-Monitoring (source recommendations 2, 5)

Patients treated for hypertension should be offered home blood pressure self-monitoring with co-interventions (Weak for). Each measurement session consists of three readings separated by at least 30 seconds after five minutes of rest; the session value is the average of the three readings. Patients transmit their readings to the care team weekly. The care team reviews the weekly average and notifies the responsible clinician when the weekly average is above goal as defined in Table 1. If a week passes without readings, a reminder is sent to the patient; after three reminder attempts on different days without readings, the care team contacts the patient directly.

3.4 Follow-up After a Therapy Change (algorithm boxes 20–23)

After drug therapy is started or changed, the patient is followed up regularly, monthly, until blood pressure is at goal, with tailored medication titration at each follow-up. The care team schedules the follow-up visit four weeks after the change and confirms that home readings are available for that visit.

Key principles:

1 Decision Tables 1 and 2 are the only decision logic in this guideline. The processes in 3.3 and 3.4 use patient reading requests, record queries, reminders, visit requests, and notifications; they do not define additional decisions.

3.5 Lifestyle (source recommendations 19–21)

At least 120 minutes per week of aerobic exercise (Strong for); a dietitian-led Dietary Approaches to Stop Hypertension diet (Strong for); and sodium intake limited to no more than 2,300 mg per day (Strong for).

4. Evidence Summary

Recommendation	Source number	Strength
Systolic goal < 130 mmHg	6	Weak for
Systolic goal < 150 mmHg, age 60 and over	7	Strong for
Systolic goal < 140 mmHg, age 60 and over with diabetes	8	Strong for
Diastolic goal < 90 mmHg, age 30 and over	9	Weak for
First-line drug classes	23	Strong for
Thiazide-type diuretic, age 65 and over	25	Weak for
No ACE inhibitor, ARB and renin inhibitor combinations	26	Strong against
Home self-monitoring with co-interventions	5	Weak for
Aerobic exercise 120 minutes per week	19	Strong for
DASH diet	20	Strong for
Sodium no more than 2,300 mg per day	21	Strong for

5. References

1 Department of Veterans Affairs, Department of Defense. VA/DoD Clinical Practice Guideline for the Diagnosis and Management of Hypertension in the Primary Care Setting, Version 5.0. March 2020.